

Primary Care Office Visit

Service Date: 03/21/2024 11:47 EDT
Status: Auth (Verified)

Northern Light Health

Patient	CANE, CANDY
DOB	07/24/1985
Gender	Female
Location	NL Primary Care — Husson Ave (Ambulatory)
Encounter Window	03/21/2024–03/23/2024
Encounter ID	379790991
Provider	Allison C. Keith, MD (performed & verified 03/21/2024 11:47)
Document Type	Primary Care Office Note (POP/Peds template)

Chief Complaint

3-month follow-up Chronic conditions

Assessment & Plan

1) Hypokalemia E87.6

Follow-up potassium levels normal.

Orders: Basic Metabolic Panel; Hemoglobin A1c (with eAG); Magnesium level; TSH with reflex to FT4; Office Visit Level 4 Established (99214; 30–39 min).

2) Hypomagnesemia E83.42

Follow-up magnesium levels normal. Uses magnesium 400 mg during diarrhea flares—reported symptomatic benefit.

Orders: Basic Metabolic Panel; Hemoglobin A1c (with eAG); Magnesium level;

TSH with reflex to FT4; Office Visit Level 4 Established (99214; 30–39 min).

3) Hypertensive disorder I10

Blood pressure elevated today. Add **losartan** back to carvedilol (Coreg) regimen. On maximal immediate-release Corg; long-acting Corg not covered.

Orders: Basic Metabolic Panel; Hemoglobin A1c (with eAG); Magnesium level; TSH with reflex to FT4; Office Visit Level 4 Established (99214; 30–39 min).

4) Pre-diabetes R73.03

Follow-up A1c planned.

Orders: Basic Metabolic Panel; Hemoglobin A1c (with eAG); Magnesium level; TSH with reflex to FT4; Office Visit Level 4 Established (99214; 30–39 min).

Vitals & Measurements

Pulse	Blood Pressure	Weight	BMI
86 bpm	170/71 → 138/71 (improved)	75 kg	29.48

Captured on 03/21/2024.

Medications

New/Active This Visit

Medication	Sig	Qty
atorvastatin 40 mg tablet	1 tab PO daily	90 tabs
carvedilol (Coreg) 25 mg tablet	1 tab PO twice daily	180 tabs
clopidogrel 75 mg tablet	1 tab PO daily	90 tabs
cyclobenzaprine 10 mg tablet	1 tab PO TID PRN muscle spasm	30 tabs
DULoxetine 60 mg DR capsule	1 cap PO daily	—
gabapentin 300 mg capsule	1 cap PO at bedtime	90 caps

Medication	Sig	Qty
hydrOXYzine pamoate 25 mg capsule	1 cap PO TID PRN anxiety	90 caps
lorazepam 0.5 mg tablet	1 tab PO TID PRN anxiety	30 tabs
losartan 25 mg tablet	1 tab PO daily (added)	90 tabs
trazodone 50 mg tablet	1–2 tabs PO HS PRN insomnia	90 tabs

Home / Unchanged

Medication	Sig
acetaminophen 500 mg (Extra Strength)	1,000 mg PO TID PRN pain
acetaminophen 650 mg	PO BID PRN pain
aspirin 81 mg	PO daily
atropine-diphenoxylate (Lomotil) 2.5/0.025 mg	2 tabs PO QID PRN loose stools
dicyclomine 20 mg tablet	1 tab PO QID PRN abdominal discomfort
Glucose monitoring supplies (Freestyle meter/strips/lancets)	Use as directed
levonorgestrel IUD (Mirena 52 mg)	Intrauterine device
lidocaine 5% patch	Apply 1 patch daily; remove after 12 hours
magnesium oxide 400 mg	PO daily
Shingrix (zoster, inactivated)	0.5 mL IM once; repeat in 2–6 months

Medications administered in office: none.

Allergies

Wellbutrin — rash

Codeine — nausea/vomiting

Lisinopril — cough

History of Present Illness

- Anxiety & MDD (moderate, recurrent): duloxetine daily; lorazepam PRN (~1 yearly script; PMP reviewed, last fill May 2022).
- Chronic dysphagia (liquids/solids) for years. GI referral placed previously for chronic diarrhea; add dysphagia to referral.

- Hypertension: BP elevated; Coreg initially effective; prior metoprolol/losartan/HCTZ stopped; infrequent home checks with high readings.
- Magnesium: normal in October; takes 400 mg during diarrhea episodes.

Review of Systems

Per HPI; no additional concerns reported.

Physical Exam

- General: awake, alert, oriented.
- Cardiac: regular rate.
- Respiratory: regular, non-labored.
- Psychiatric: appropriate mood/affect; good judgment and insight.

Orders Summary

Order	Purpose / Notes
Basic Metabolic Panel	Electrolytes (K+, Mg2+ status)
Hemoglobin A1c (with eAG)	Prediabetes follow-up
Magnesium level	Monitor hypomagnesemia
TSH with reflex to FT4	Thyroid screening if abnormal
Office Visit Level 4 Est (99214)	30–39 minutes

Treatment Notes

- Restart losartan with carvedilol for BP control; long-acting Coreg not covered.
- Continue anxiety, depression, sleep, and pain regimens as appropriate; monitor controlled substance use.

- Add dysphagia to GI referral; evaluate chronic diarrhea pattern and magnesium use.
- Encourage structured home BP log; bring to next visit.

Sign-Off

Electronically signed by Allison C. Keith, MD on 03/21/2024 11:47 AM.